

Urethroplasty (Urethral Repair)

Patient Information · Surgery to repair a narrowing of the urethra

WARWIKI

Urethroplasty is surgery to repair a narrowed, scarred part of the urethra. The urethra is the tube that carries urine out of your body. The narrowing (a stricture) slows or blocks your urine stream. This surgery opens it back up and gives the longest-lasting repair.

About This Procedure

A **stricture** is a scarred, narrow spot in the urethra. It makes it hard to empty your bladder. Stretching or cutting the scar through a scope often works only for a while. Urethroplasty **rebuilds** the urethra to a normal width. How it is done depends on where the narrowing is and how long it is:

- The **cut** may be on the **penis**, in the area between the scrotum and anus (the **perineum**), or both.
- A **short** narrowing can be **removed and the healthy ends sewn together**.
- A **longer** narrowing is **widened with a graft**. This is often a patch from inside your cheek (it has its own handout).
- On the **penis**, a badly scarred urethra, for example from lichen sclerosus, may be repaired in **two operations**, usually at least 4 to 6 months apart.

Your surgeon will tell you which repair fits your stricture.

Is It Safe?

Urethroplasty is a well-established operation. In studies, more than 8 in 10 repairs for longer strictures stay open, but results vary with the type of repair. Like any surgery, it has risks: bleeding, infection, or the narrowing coming back over time. A narrowing that comes back can usually be treated again.

Some men notice changes in erections or ejaculation, a slightly shorter penis, or a softer tip of the penis during erection. These changes are uncommon and often improve, but not always. Ask your surgeon about your own risk.

After a Pelvic Fracture

A broken pelvis can tear the urethra deep behind the scrotum. This injury is repaired differently. A tube through the lower belly (a **suprapubic catheter**) drains urine first. The repair is usually done **at least 3 months** after the injury, once other injuries have healed. X-rays with dye from above and below show the gap, and the healthy ends are then joined. About 9 in 10 of these repairs stay open in large studies.

About 1 in 3 men have erection problems from the injury itself, before any repair. Leaking urine after the repair is less common (fewer than 1 in 10 in a large study). Your team checks the valve at the bladder outlet beforehand, because it helps keep you dry.

Learn the Terms

Urethra

The tube that carries urine out of the body.

Stricture

A scarred, narrow spot in the urethra that slows or blocks the urine stream.

Urethroplasty

Surgery to repair the narrowing and rebuild the urethra to a normal width.

Graft

A small patch of your own tissue (often from inside the cheek) used to widen the urethra.

Perineum

The area between the scrotum and the anus, where one common cut is made.

Catheter

A soft tube left in the urethra to drain urine while the repair heals.

Urethrogram

An X-ray of the urethra with dye. It checks that the repair has healed before the catheter comes out.

Cystoscopy

A look inside the urethra with a thin camera.

Will it hurt? You will be asleep or numb from the waist down, so you feel nothing during surgery. Afterward, expect soreness, swelling and bruising near the cut for a couple of weeks. Ice, snug underwear and pain medicine help.

First days Soreness and swelling. You go home with a catheter.	Weeks 2–3 Often an X-ray check, then the catheter comes out. Timing varies.	First year Visits to check your stream. Often a camera check at about 3 months.	After that Tell your team if your stream slows again, even years later.
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How to Get Ready

- Follow your surgery instructions about eating, drinking and which medicines to stop, including blood thinners.
- A **urine test** checks for infection. Any infection is treated first.
- Stop smoking.** It slows healing.
- If you self-dilate, have a catheter, or had a recent stretch or cut, surgery is often planned **at least 3 months** after the last one so the urethra can rest. A tube through the lower belly may drain urine meanwhile.
- Plan for a **catheter** for a while after surgery, and arrange a ride home.

Tell your team ahead of time if you:

- Take a **blood thinner**, or have a urine or skin infection
- Have had urethral surgery, radiation, or a skin condition like lichen sclerosus

What Happens During Surgery

- You get general or spinal anesthesia and antibiotics.
- The surgeon makes a cut over the narrowing and opens the scarred part.
- The narrowing is removed and the ends joined, or it is widened with a graft.
- A **catheter** is placed to drain urine and protect the repair while it heals.

After Surgery

- The catheter usually stays about 2–3 weeks. Do not pull on it.
- Swelling, bruising and some blood-tinged urine around the catheter are normal early on.
- Avoid heavy lifting, straining, cycling and sex until your team says it is safe.

Call the office that did your surgery if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- Redness, swelling or pain that is getting worse, or fluid leaking from the cut
- A catheter that falls out. Do not put it back in yourself.

Go to an emergency department if:

- The catheter stops draining, or you cannot urinate after it is out, and your bladder feels full or painful
- Heavy bleeding, or urine like red wine or with clots
- You cannot reach the office and are worried

Three Things to Remember

- Urethroplasty rebuilds a narrow urethra and gives the longest-lasting repair.
- Keep the catheter in until your team removes it, usually after an X-ray check.
- A narrowing can come back. Tell your team if your stream slows again.